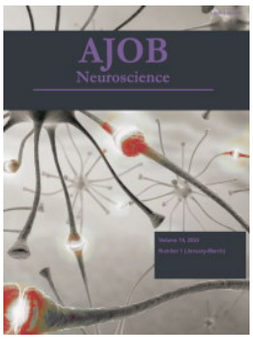

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Preferences of Individual Mental Health Service Users Are Essential in Determining the Least Restrictive Type of Restraint

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might also be permissible when the risk of inadequate chemical restraint might pose a threat to the wellbeing of the milieu. A relational approach encompasses our colleagues' central claim that autonomy should be preserved where possible but expands the conditions for the permissible use of chemical restraints to more readily address principle conflicts which arise in practice.

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Preferences of Individual Mental Health Service Users Are Essential in Determining the Least Restrictive Type of Restraint

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Crutchfield and Redinger (2024) propose that the use of a chemical restraint that affects only a particular conscious state is ethically permissible if, and only if, (1) it is the least restrictive means to avert harm, (2) it is medically appropriate, (3) one has obtained the person's informed consent, and (4) it promotes the person's decision-making capacity. We find it laudable that Crutchfield and Redinger set out to provide ethical clarity in a controversial area of practice characterized by a dearth of empirical evidence and ethical analysis.

Our comment focuses on the use of restraints in psychiatric settings. Based on Pettit's conception of basic liberty, Crutchfield and Redinger argue that chemical restraints that affect only particular conscious states are less restrictive than physical restraints. They conclude that their account "would support the use of chemical restraints over the use of

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physical restraints" (2024, 14) in psychiatric settings. We will show that several factors complicate the application of the proposed criteria in psychiatric settings and that a generic ethical prioritization of chemical over physical restraint cannot be upheld. We will contend that decisions about restraint measures in psychiatric settings must be made on a case-by-case basis guided by the preferences of individual service users.

We will focus exclusively on chemical restraints that affect a particular conscious state and set aside chemical restraints that affect whether a person is conscious. Whenever we use the term "chemical restraint," it should thus be understood in the former way. We will use the term "physical restraint" as encompassing both holding and mechanical restraint but not seclusion (i.e., isolating a person in a designated room). Holding involves trained mental health professionals physically holding a person to restrict their movement, and mechanical restraint involves the use of straps, belts, or other devices to restrict a person's movement.

The application of Crutchfield and Redinger's criteria in psychiatric settings is complicated by several factors. First, turn to criterion (4), the requirement that chemical restraints promote decision-making capacity. Medications used as chemical restraint to control agitation or aggression in psychiatric settings (e.g., benzodiazepines or olanzapine at higher dosages) often have sedative effects, and these sedative effects compromise decision-making capacity (Maeck and Stoppe 2010). Reducing medication with sedative effects is therefore recommended as a means to promote decision-making capacity in the literature on supported decision-making (Scholten and Gather 2018). Crutchfield and Redinger could reply that chemical restraint may promote decision-making capacity indirectly, namely, by reducing agitation and aggression, which are both likely to impact decision-making capacity negatively. In such cases, however, positive effects on decision-making capacity, if they occur at all, are but a fortunate side effect of the use chemical restraint.

Next, consider criterion (3), the requirement that one has obtained the person's informed consent for the use of a chemical restraint. This criterion does not provide much guidance in psychiatric settings, as chemical restraint is typically used in situations where service users are unable to give consent due to agitation, aggressiveness, or other psychiatric symptoms. Conceding that obtaining informed consent will often not be possible, the authors propose that such situations count as emergency situations that are exempt from the consent requirement. This solution gives up the ideal of informed consent a bit too easily. When service users are unable to give consent in ordinary clinical practice, recourse is taken to substituted decision-making according to the so-called substituted judgment standard. We would argue that similar considerations apply to decisions about coercive measures, at least in cases of harm to self: if a service user is not able to give consent, the restraint measure that should be used is the one that the service user would have preferred had she had the ability to give consent. Moreover, Crutchfield and Redinger seem to appreciate insufficiently what the absence of consent means for the phenomenology of chemical restraint. If a service user refuses to take oral medication, chemical restraint has to be carried out by using physical restraint, at least for a short duration, and administering medication via (typically intramuscular) injection. The phenomenology of this procedure may not be discarded in ethical analysis.

Now proceed to condition (2), the requirement that the use of chemical restraints be medically appropriate. Systematic reviews on seclusion and physical restraint indicated that while the benefits of physical restraints remain unclear, they have detrimental physical and psychological effects, including injuries, deep-vein thrombosis, cardiac arrest, post-traumatic stress disorder, and revival of previous trauma (Chieze et al. 2019; Kersting, Hirsch, and Steinert 2019). A systematic review on chemical restraints concluded that "the drugs tested […] generally rapidly reduce the symptoms of concern" (Muir-Cochrane et al. 2020, 120), yet the included studies also reported adverse events in 0 to 38.1% of cases. Adverse events included physiological effects like respiratory depression, vomiting, akathisia, dystonia, and seizures as well as psychological effects like extreme sedation, amnesia, and confusion. This evidence casts doubt on whether restraint measures can ever legitimately be called "medically appropriate." Recognizing the importance of comparing the risk-benefit ratios of the available options, we have therefore proposed proportionality as an alternative concept in the ethical evaluation of restraint measures (Gather et al. 2020).

Lastly, consider criterion (1), the requirement that chemical restraint be the least restrictive alternative. Acknowledging that this criterion also applies to physical restraint, Crutchfield and Redinger support their ethical prioritization of chemical over physical restraint with the claim that in the case of chemical restraints, "the situations in which [the] conditions

are satisfied will be more frequent" because chemical restraints are less liberty-intrusive (2024, 10).

This claim is premised on a specific conception of basic liberty. Crutchfield and Redinger understand the liberty-intrusiveness of a restraint measure in terms of the extent to which it restricts basic liberty as a liberty "that is required for living the life of a free person" (2024, 6). Since the authors hold that chemical restraints generally restrict this kind of liberty less than physical restraints do, they conclude that chemical restraints should generally be prioritized over physical restraints. Note, however, that one could just as well arrive at the opposite conclusion, if only one assumes a different conception of basic liberty. In several landmark judgments, the German Federal Constitutional Court understood basic liberty as including both bodily integrity and freedom of movement, with the former ranked higher than the latter. The court reasoned that involuntary medication by means of intramuscular or intravenous injection is invasive and hence violates bodily integrity, while physical restraint only restricts freedom of movement. It accordingly concluded that physical restraint is the less restrictive alternative in comparison to involuntary medication (Henking and Scholten 2023). This reasoning would also apply to chemical restraint by means of injection.

We support neither of these arguments. Rather, we endorse Georgieva, Mulder, and Wierdsma's (2012, 11) claim that "while no objective scientific evidence from controlled studies" – and we want to add: no agreed-upon conception of basic liberty – "can guide us into choosing the least restrictive and the most effective measure, the only rational way to choose the correct intervention is to consider patients' individual choices." Empirical research on service users' preferences regarding types of restraint measures showed high variation among individual service users (Georgieva, Mulder, and Wierdsma 2012). Rather than trying to establish a generic ethical priority of one type of restraint over another, it would be more helpful to look at ways in which the preferences of individual service users can be brought to the foreground.

Psychiatric advance directives provide a unique opportunity for doing so. Psychiatric advance directives are documents that service users can use to express their preferences for treatment during a future mental health crisis. A systematic review on the content of psychiatric advance directives carried out by our research group showed that in 36% ($n = 15$ of $N = 42$) of the studies included, service users stated preferences regarding the type of coercive measure to be used in cases where coercion is unavoidable (Gaillard et al. 2023). These preferences can guide decision-making about restraint measures. Reasons why a sizeable proportion of service users preferred other coercive measures over chemical restraint included, for example, previous experience with severe side effects of medication and fear of needles.

There are other ways to open the discussion about service users' preferences regarding coercive measures if no advance directive exists. Debriefing after the use of coercion provides such occasion. Looking for alternative ways to give center stage to the preferences of service users is important because completion rates for psychiatric advance directives are still low (Braun et al. 2023). At the same time, the fact that psychiatric advance directives can guide decision-making about restraint measures is yet another reason to promote their implementation.

Crutchfield and Redinger make transparent that their claim that chemical restraints are ethically preferable over physical restraints only holds "other things being equal" (2024, 10). Still, the attempt to establish an ethical prioritization of this kind could distract from the fact that clinical decisions about restraint measures should be guided by the preferences of individual service users. Our argument once more underlines the urgent need to discuss and respect the preferences of service users in psychiatric settings.

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Chemical Restraints and the Basic Liberties

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Crutchfield and Redinger (2024) argue that, ceteris paribus, it is morally worse to deploy a restraint that undermines a basic liberty than one that does not.[1] This is a plausible view, and is likely to be shared by many. Their account of what constitutes a basic liberty is more controversial. They hold that physical restraints – such as those that prevent a patient from moving his arms and legs during a medical procedure – and chemical restraints that render a person unconscious, both intrude upon a person's basic liberty. Whereas chemical restraints that change a person's particular conscious state, without rendering him unconscious, do not intrude upon a basic liberty. As a result, they conclude "the *first line restraint should be chemical restraints that only change the particular conscious state the person is in*" (13, italics in original). While this has considerable importance in determining how we ought to treat patients in medical contexts, it also has implications for the debate in criminal justice on the use of neurointerventions—"interventions that exert a physical, chemical or biological effect on the brain in order to diminish the likelihood of some forms of criminal offending" (Douglas and Birks 2018, 2). For if an interference with a particular conscious state does not intrude upon a basic liberty, then we could potentially circumvent one objection to their use (Bublitz and Merkel 2014).

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[1]The ceteris paribus clause is important because Crutchfield and Redinger make it clear that other considerations matter too. They write "just because a restraint is more intrusive than an alternative, it doesn't follow that it is less permissible" (2024, 12).

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